



Non-Obstetric Related Admission of Pregnant Women >20 Weeks Gestation Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Health Service	Obstetrics and Gynaecology	Midwifery and Medical

2. Purpose

Pregnant women requiring admission to hospital for non-obstetric reasons may have issues arise that can lead to pregnancy complications, or their pregnancy may contribute or exacerbate an underlying condition.

This procedure outlines the process for the development of a management plan for women presenting to the hospital whilst pregnant >20 weeks gestation.

3. Principles of management

- Women >20 weeks gestation should be co-managed with the Consultant of the admitting specialty and a Consultant Obstetrician.
- Location of admission should be determined on a case-by-case basis; and is dependent on the admission diagnosis and bed availability within the hospital.
- In the event of clinical deterioration, planning around timing, mode and place of birth needs to be agreed by a multidisciplinary team approach. This should involve the admitting specialty Consultant, Consultant Obstetrician, Paediatric Consultant, Anaesthetic Consultant, Intensive Care Unit (ICU) Consultant, Midwife Manager and/or Associate Midwife Manager.
- In the event of a woman presenting <35 weeks gestation in labour or if the fetus is in distress, every effort should be made for in-utero transfer to a tertiary hospital. However, if the mother is critically unwell and inter-hospital patient transfer is not in the woman's best interests, then immediate delivery should be undertaken at Armadale Health Service (AHS) regardless of gestation.
- If the woman is an ICU patient at AHS, any transfer to another site for birth must include assessment of ICU capability at the receiving site.
- Transfer of the critically unwell woman should occur with medical staff escort that are able to manage acute deterioration (including advanced airway management) during transfer.
- Care planning discussions are to include the woman (and support person) with consideration given to culturally safe care delivery¹. Strategies may include:
 - Utilising [Interpreter and Translation Services](#)
 - Acknowledging the woman's cultural preferences
 - Positioning – sitting alongside, not opposite, quiet or shy women and families
 - Ensuring privacy and confidentiality
 - Involving her partner/the father of the baby, where this is agreed by the woman.

4. Escalation of Care

- In the event that a woman referred for internal specialty review is not seen by the Admitting Registrar within the one hour timeframe post-referral then this is to be escalated to the relevant on-call internal specialty Consultant to expedite review.
- If no resolution is able to be reached then the matter is to be escalated to the Head of Department Obstetrics and Gynaecology for discussion with internal specialty teams to attend to review the patient and arrange a transfer to the appropriate inpatient ward as per [Appendix A](#).
- In the event of any conflict of clinical opinion the [Escalation if Clinical Conflict Guideline](#) is to be followed.
- For any acute deterioration, escalation of care is to be carried out in accordance with the escalation protocols on the Maternity Adult Deterioration Detection System (MADDS) chart.

5. Roles and responsibilities

Role	Responsibility
Emergency Department (ED) Triage Nurse	• Complete a primary survey of the pregnant woman who presents to the ED triage window and will allocate a triage score • Contact the AAU if the presenting problem is thought to be obstetric in nature and complete a clinical handover prior to sending the patient up to the AAU.
Antenatal Assessment Unit (AAU) Registered Midwife	• Assess the woman on presentation to the AAU and complete a set of physiological observations on the woman; documenting the results on the MADDS chart • Escalate care of the woman in accordance with the escalation criteria documented on the MADDS chart • Contact the appropriate medical officer following assessment to support timely development of a management plan.
General Practitioner Obstetrician	• Review the woman and if the condition is not obstetric in nature contact the relevant admitting Registrar to review the patient in the AAU • Document a plan of management until admitting Registrar has reviewed the woman and accepted clinical governance.
Specialist Obstetrician	• Review the woman and make a plan of management. If the presenting complaint is not obstetric in nature contact the admitting Registrar. • Document a plan of management until admitting Registrar has reviewed the woman and accepted clinical governance.
Admitting Registrar	• Review the woman on AAU within one hour of the woman being referred; to develop an action plan and/or arrange transfer to the appropriate inpatient ward.

6. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy \(MP0095/18\)](#)
- HDWA [Admission Policy \(MP0058/17\)](#)
- HDWA [Cardiotocography Monitoring Policy \(MP0076/18\)](#)
- East Metropolitan Health Service (EMHS) [Consent Policy \(EMHS: 5\)](#)
- EMHS [Fetal Surveillance and Cardiotocography Monitoring Policy \(EMHS: 195\)](#)
- EMHS [Discharge Policy \(EMHS: 99\)](#)

7. Supporting information

The following documents support the implementation of this procedure:

- [Fetal Surveillance Guideline \(OBS-GUI-0172-15\)](#)
- [Interpreter and Translator Guidelines \(AKG-GUI-0212-00\)](#)
- [Clinical Conflict Escalation Policy \(OBS-GUI-0160-15\)](#)

8. Compliance, monitoring and evaluation

Compliance against this procedure will be monitored by the Obstetrics and Gynaecology Safety and Quality Committee.

A register of women attending the AAU will be commenced with biannual auditing of the registers contents to be incorporated into the Obstetrics and Gynaecology audit program.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.7 Planning for comprehensive care

The health service organisation has processes relevant to the patients using the service and the services provided:

- a. For integrated and timely screening and assessment
 - b. That identify the risks of harm in the 'Minimising patient harm' criterion
- Standard 6 – Action 6.7 Clinical handover

The health service organisation, in collaboration with clinicians, defines the:

- a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover
- Standard 8 – Action 8.3 Partnering with consumers

Clinicians use organisational processes from the Partnering with Consumers Standard when recognising and responding to acute deterioration to:

- Actively involve patients in their own care
 - b. Meet the patient's information needs

- c. Share decision-making
- Standard 8 – Action 8.4 Recognising acute deterioration

The health service organisation has processes for clinicians to detect acute physiological deterioration that requires clinicians to:

- a. Document individualised vital sign monitoring plans
- b. Monitor patients as required by their individualised monitoring plan
- c. Graphically document and track changes in agreed observations to detect acute deterioration over time, as appropriate for the patient
- Standard 8 – Action 8.6 Escalating care

The health service organisation has protocols that specify criteria for escalating care, including:

- a. Agreed vital sign parameters and other indicators of physiological deterioration
- b. Agreed indicators of deterioration in mental state
- c. Agreed parameters and other indicators for calling emergency assistance
- d. Patient pain or distress that is not able to be managed using available treatment
- e. Worry or concern in members of the workforce, patients, carers and families about acute deterioration

10. References

1. Department of Health (2018) Clinical Practice Guidelines: Pregnancy Care. Canberra: Australian Government, Department of Health.

11. Policy contact

HOD Obstetrics &Gynaecology

Enquiries relating to this procedure may be directed to:

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12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	01/05/2023	01/05/2026	Scheduled review

13. Authorisation

This procedure has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Alison Parr – Director Clinical Services
Authorisation date	28/04/2023
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Appendix A – Unplanned obstetric presentations (>20 weeks gestation) to the AHS Emergency Department

